

Patient 15

Report A

653714.420/06/2016 CT Acute Stroke

For Thrombolysis? No

indication: Receptive and expressive aphasia. Onset time uncertain. No fever. Hypertensive 180/110,

Possible left Cerebellar infarct

CT HEAD

Haemorrhage: No

Intravascular thrombus: No

Acute Infarct: Left parieto-occipital lobe subacute infarct with localised mass effect demonstrating sulcal effacement, no midline shift. Other findings and comments: Multiple low attenuating foci in the right frontal and parietal centrum ovale, subinsular white matter and both basal ganglia are consistent with deep white matter lacunes, on a background of periventricular white matter low attenuation consistent with mild chronic Small vessel disease.

CT ANGIOGRAM Aortic arch and subclavian arteries: Bovine configuration of the aortic arch, with the right common carotid artery arising from the brachiocephalic trunk.

RCCA/ICA: Calcified atheroma of the right cavernous ICA causes arterial narrowing but no significant stenosis.

LCCA/ICA: Scattered non-stenosing calcification of the left supraclinoid ICA.

RVA: Non dominant. Significant narrowing of V4 intradural segment, although distal opacification is seen suggesting patency.

LVA. Dominant, non stenosing calcification at the origin arising from left subclavian artery. Significant stenosing mixed atheroma of the intradural segment, although distal opacification suggests patency. intracranial Arteries: Proximal branches of the circle of Willis are patent, no intracranial aneurysms, dissection, occlusion or thrombus,

Fetal left PCA noted.

Other findings and comments: Centrilobular emphysema at both apices, otherwise visualised lungs are clear. No destructive focal bone lesion.

Conclusion: Subacute left parieto-occipital lobe infarct. Significant stenosing mixed atheroma of the intradural vertebral arteries bilaterally, although distal opacification indicates patency of both vessels. No proximal intravascular thrombus. Embolic aetiology should be considered - echocardiogram and MRI head would help to further evaluate.

Report B

6537825 22/06/2016 MR Head

Clinical history: Receptive and expressive aphasia. Onset time uncertain. LMCA infarct. 2 mechanism

Findings:

Comparison has been made with CT head 20/06/2016.

There is diffusion restriction with corresponding low ADC values located in the left inferior 1 superior parietal lobule, parietal operculum and dorsal aspect of the left insular cortex with mild surrounding mass effect - consistent with recent infarction. A couple of tiny gyral micro haemorrhages are shown related to this region.

The proximal branches at the Circle of Willis are patent with no focal stenosis, occlusion or acute dissection. Foetal left PCA. Right posterior communicating artery not visualised.

There are further multifocal areas of micro haemorrhage located throughout the basal ganglia, thalami, midbrain, pons, cerebellum and both temporal / right occipital lobes.

There is periventricular and deep white matter hyperintensities in keeping with mild to moderate chronic small vessel ischaemia.

Generalised cerebral sulcal prominence normal for age.

Comment: Acute/subacute left MCA territory infarct with mild surrounding mass effect, no complication of parenchymal haemorrhage. There is multifocal predominantly central areas of microhaemorrhage scattered throughout the cerebrum and cerebellum. Whilst the central microhaemorrhages involving the deep grey matter nuclei would suggest a hypertensive microangiopathy, the more peripheral foci would be more compatible with amyloidosis.